EMERGENCY SURGERY · MIDTERM · COMPLETE QUESTION BANK

Emergency Surgery — Midterm All Lectures Question Bank

جراحة / طب الطوارئ — منهج الميدترم — مجمّع أسئلة المادة كلها

4	11	198	59
Lectures	Past exam questions	Expected Qs	Flashcards

#	Lectures included in this file · المحاضرات اللي في الملف	Past Qs	Expected	Cards
1	Acute Abdomen البطن الحادة	1	59	15
2	Acute Pancreatitis التهاب البنكرياس الحاد	4	39	14
3	Burn الحروق	2	50	15
4	Trauma الإصابات (Trauma)	4	50	15

المصادر:

كويز سابق (جراحة طوارئ)

ترقيم الأسئلة: رقم المحاضرة ثم رقم السؤال (مثلاً 4.12). الأسئلة المتوقعة عليها حرف N. الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

LECTURE 1

Acute Abdomen

البطن الحادة

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1.1 The most common site of peptic ulcer is: كويز سابق (جراحة طوارئ)

a. 1st part of duodenum
b. Lesser curvature
c. Fundus of stomach
d. Distal end of esophagus

Part B · Expected Questions (from the lecture)

أسئلة متوقعة على المحاضرة بنفس أسلوب الكويزات

1.N1 The term acute abdomen refers to: MCQ

a. Abdominal mass
b. Acute abdominal pain
c. Abdominal distension only
d. Chronic abdominal pain

1.N2 To reach a proper diagnosis of acute abdomen, all of the following are needed EXCEPT: MCQ

a. Immediate laparotomy for all patients
b. Laboratory and radiological investigations
c. Complete physical examination
d. Careful history taking

1.N3 All of the following are upper abdominal causes of acute abdomen EXCEPT: MCQ

a. Colonic diverticulitis
b. Perforated peptic ulcer
c. Acute pancreatitis
d. Acute cholecystitis

1.N4 Acute myocardial infarction may present as upper abdominal pain. TRUE / FALSE

a. True
b. False

1.N5 Which of the following is a cause of right lower abdominal pain? MCQ

a. Mesenteric adenitis
b. Colonic diverticulitis
c. Perforated peptic ulcer
d. Leaking aortic aneurysm

1.N6 Colonic diverticulitis is a cause of: MCQ

a. Upper abdominal pain
b. Right lower abdominal pain
c. Left lower abdominal pain
d. Pelvic pain only

1.N7 All of the following are medical causes of acute abdominal pain EXCEPT: MCQ

a. Familial Mediterranean fever
b. Diabetic ketoacidosis
c. Uremia
d. Acute appendicitis

1.N8 Mesenteric vascular occlusion can cause both upper and mid-abdominal pain. TRUE / FALSE

a. True **b.** False

1.N9 The most common acute abdominal surgical emergency is: MCQ

a. Acute cholecystitis **b.** Intestinal obstruction
c. Acute appendicitis **d.** Perforated peptic ulcer

1.N10 Acute appendicitis is commonest in: MCQ

a. The second and third decades **b.** The fifth decade
c. Infants **d.** The elderly

1.N11 Appendicitis is rare in infants because the appendix is: MCQ

a. Full of lymphoid tissue **b.** Wide-mouthed and well drained
c. Narrow and obstructed **d.** Retrocecal

1.N12 Obstructive acute appendicitis represents about: MCQ

a. 10% of cases **b.** All cases
c. 1/3 of cases **d.** 2/3 of cases

1.N13 The usual cause of obstruction of the appendix lumen is: MCQ

a. Faecolith **b.** Worms
c. Tumor **d.** Foreign body

1.N14 Non-obstructive appendicitis is more likely to perforate than obstructive appendicitis. TRUE / FALSE

a. True **b.** False

1.N15 An appendicular mass usually forms within: MCQ

a. 24 hours **b.** 3–5 days
c. 3 weeks **d.** 3 months

1.N16 Portal pyaemia as a complication of appendicitis means: MCQ

a. Peritonitis
b. Thrombophlebitis of the portal venous system with hepatic abscess
c. Intestinal obstruction
d. Pelvic abscess

1.N17 All of the following are possible courses of acute appendicitis EXCEPT: MCQ

a. Malignant transformation **b.** Appendicular mass
c. Gangrene **d.** Resolution

1.N18 The scoring system used in diagnosis of acute appendicitis is: MCQ

a. Child score **b.** Alvarado score (MANTRELS)
c. Glasgow score **d.** Ranson score

1.N19 **In the Alvarado score, which items score 2 points?** MCQ

a. Migration of pain and fever
b. Anorexia and nausea
c. Rebound and shift to the left
d. Tenderness in the right lower quadrant and leucocytosis

1.N20 **In acute appendicitis, vomiting always occurs before pain.** TRUE / FALSE

a. True **b.** False

1.N21 **The pain of acute appendicitis characteristically:** MCQ

a. Is in the left iliac fossa
b. Starts around the umbilicus then shifts to the right lower abdomen
c. Starts in the right iliac fossa then becomes generalized
d. Starts in the back

1.N22 **Absence of leucocytosis excludes acute appendicitis.** TRUE / FALSE

a. True **b.** False

1.N23 **Rovsing's sign is pain in the right iliac fossa on:** MCQ

a. Deep palpation of the left lower quadrant
b. Coughing
c. Extension of the right thigh
d. Pressing on the right hypochondrium

1.N24 **Psoas sign is elicited by:** MCQ

a. Deep palpation of the left iliac fossa
b. Flexion of the left hip
c. Extension of the right thigh with the patient lying on the left side
d. Rectal examination

1.N25 **Rigidity in a patient with appendicitis suggests:** MCQ

a. Appendicular mass **b.** Perforation
c. Mesenteric adenitis **d.** Resolution

1.N26 **Appendicitis in the elderly is more likely to perforate because of:** MCQ

a. Prominent vomiting
b. Well developed omentum
c. Little tenderness, late diagnosis and low immunity
d. Wide lumen

1.N27 **In infants and young children appendicitis is dangerous because of all EXCEPT:** MCQ

a. Greater omentum not well developed
b. Difficult examination
c. Prominent vomiting so misdiagnosed as enteritis
d. Pain displaced upwards

1.N28 **In pregnancy, appendicitis may be misdiagnosed as:** MCQ

a. Ectopic pregnancy only
b. Pyelitis
c. Cholecystitis
d. Gastroenteritis

1.N29 **Mesenteric adenitis is characterized by:** MCQ

a. Fixed tenderness
b. Tenderness that shifts with change of position, usually in children
c. Occurring only in elderly
d. High amylase

1.N30 **Meckel's diverticulitis is usually diagnosed:** MCQ

a. By CT
b. By ultrasound
c. After exploration
d. By blood tests

1.N31 **Diagnosis of acute appendicitis is essentially:** MCQ

a. Radiological
b. By laboratory tests only
c. By laparoscopy only
d. Clinical

1.N32 **Appendectomy should not be delayed especially in all of the following EXCEPT:** MCQ

a. Young healthy adults with appendicular mass
b. Pregnant
c. Elderly
d. Children

1.N33 **The treatment of appendicular mass is:** MCQ

a. Ochsner–Sherren regimen then interval appendectomy after 3 months
b. Immediate appendectomy
c. Discharge without follow-up
d. Laparotomy and drainage

1.N34 **The treatment of appendicular abscess is:** MCQ

a. Antibiotics only
b. Observation
c. Sonar-guided aspiration, surgical drainage if failed
d. Immediate appendectomy

1.N35 Calcular cholecystitis represents about: MCQ

a. 50%
b. 75%
c. 98%
d. 10%

1.N36 Acute cholecystitis is caused by: MCQ

a. Obstruction of the common bile duct
b. Obstruction of the cystic duct or neck of the gallbladder by a stone
c. Pancreatitis
d. Bacterial infection without stones

1.N37 The pain of acute cholecystitis lasts: MCQ

a. Less than 3 hours
b. More than 6 hours
c. A few seconds
d. Less than 1 hour

1.N38 Positive Murphy's sign is characteristic of: MCQ

a. Acute cholecystitis
b. Acute appendicitis
c. Diverticulitis
d. Acute pancreatitis

1.N39 In acute cholecystitis liver function tests are usually: MCQ

a. Markedly elevated
b. Show high ammonia
c. Show low albumin
d. Normal

1.N40 Ultrasound findings of acute cholecystitis include all EXCEPT: MCQ

a. Air under the diaphragm
b. Gall stones
c. Thickened wall
d. Pericholecystic fluid collection

1.N41 Early cholecystectomy is recommended within _____ of the acute attack: MCQ

a. 3 days
b. 3 weeks
c. 3 months
d. 6 weeks

1.N42 If a patient with acute cholecystitis deteriorates during conservative treatment, the treatment is: MCQ

a. Continue antibiotics
b. ERCP only
c. Surgery: cholecystectomy or cholecystostomy
d. Discharge

1.N43 Perforated peptic ulcer may be related to intake of: MCQ

a. NSAIDs or corticosteroids
b. Antacids
c. Iron
d. Antibiotics

1.N44 The ratio of duodenal to gastric perforated ulcer is: MCQ

a. 1:1
b. 6:1
c. 1:6
d. 10:1

1.N45 Board-like rigidity is characteristic of: MCQ

a. Acute pancreatitis
b. Mesenteric adenitis
c. Perforated peptic ulcer
d. Uremia

1.N46 The best investigation for perforated peptic ulcer is: MCQ

a. Barium enema
b. Abdominal ultrasound
c. Serum lipase
d. Erect chest X-ray showing air under the diaphragm

1.N47 The surgical treatment of perforated duodenal ulcer is: MCQ

a. Cholecystectomy
b. Graham's omental patch
c. Whipple operation
d. Hartmann procedure

1.N48 The aetiology of diverticular disease is thought to be: MCQ

a. Infection
b. Lack of fibre leading to chronic constipation and increased intracolonic pressure
c. Genetic only
d. High fibre diet

1.N49 The commonest site of colonic diverticulitis is: MCQ

a. Transverse colon
b. Caecum
c. Sigmoid colon
d. Rectum

1.N50 Barium enema is the investigation of choice during an acute attack of diverticulitis. TRUE / FALSE

a. True
b. False

1.N51 Diverticulitis with generalized peritonitis is treated by: MCQ

a. Exploration with Hartmann's procedure
b. U/S guided drainage
c. Graham's patch
d. Conservative treatment

1.N52 Mesenteric ischemia is a type of: MCQ

a. Simple intestinal obstruction
b. Paralytic ileus
c. Strangulation intestinal obstruction
d. Peptic ulcer

1.N53 Mesenteric arterial embolism may be caused by: MCQ

a. Peptic ulcer
b. Atrial fibrillation or mural thrombus after MI
c. Pancreatitis
d. Hypertension

1.N54 Mesenteric ischemia is characterized by: MCQ

a. Chronic pain only
b. Minimal symptoms with maximum signs
c. No pain
d. Maximum symptoms with minimal signs

1.N55 The investigation that shows the cause and status of the bowel in mesenteric ischemia is: MCQ

a. Barium meal
b. Plain X-ray
c. Ultrasound
d. CT angiography

1.N56 In mesenteric ischemia with gangrenous bowel, a second look operation is done after: MCQ

a. 6 hours
b. 24 hours
c. 1 week
d. 1 month

1.N57 Mesenteric venous thrombosis with viable bowel is treated by: MCQ

a. Bypass graft
b. Anticoagulation therapy
c. Resection
d. Embolectomy

1.N58 Plain X-ray of the abdomen in acute abdomen mainly detects: MCQ

a. Pancreatitis
b. Gallstones
c. Stones of the urinary tract
d. Ovarian torsion

1.N59 Abdominal ultrasound can diagnose all of the following EXCEPT: MCQ

a. Ruptured ovarian cyst or ovarian torsion
b. Renal stones
c. Acute calcular cholecystitis
d. Air under the diaphragm

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Acute abdomen	Acute abdominal pain needing urgent diagnosis: careful history, full exam, labs and imaging.	**Upper abdominal causes**	Perforated peptic ulcer, biliary colic/cholecystitis, acute pancreatitis, leaking AAA, MI.
RIF causes	Appendicitis, mesenteric adenitis, right ureteric colic, right ovarian cyst rupture/torsion.	**LIF cause**	Colonic diverticulitis (sigmoid), left ureteric colic.
Medical causes	DKA, MI, uremia, gastroenteritis, familial Mediterranean fever.	**Appendicitis**	Most common surgical emergency; peak 2nd–3rd decades; faecolith obstruction in 2/3.
Appendicitis course	Resolution, gangrene, perforation → peritonitis, appendicular mass (3–5 days), abscess, portal pyemia.	**Appendicitis signs**	McBurney tenderness, rebound, Rovsing's, psoas sign; Alvarado score (MANTRELS).
Appendicular mass	Ochsner–Sherren conservative regimen → interval appendectomy after 3 months.	**Acute cholecystitis**	98% calculus; RUQ pain > 6 h → right shoulder; Murphy's sign; US first.

Cholecystitis treatment	Early cholecystectomy within 3 days, or conservative then surgery after 6 weeks.
Peptic ulcer site	Most common: 1st part of the duodenum (duodenal : gastric = 6 : 1).
Mesenteric ischemia	Pain out of proportion to signs; CT angiography; urgent surgery (embolectomy / bypass / anticoagulation).
Perforated peptic ulcer	Sudden epigastric pain, board-like rigidity, air under diaphragm; Graham's omental patch.
Diverticulitis	Low-fiber diet; sigmoid; CT with contrast; NPO, IV fluids, antibiotics; Hartmann if peritonitis.

Answer Key — All Lectures

الإجابات

1.1. a — 1st part of duodenum
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

1.N1. b — Acute abdominal pain
Slide 2.

1.N2. a — Immediate laparotomy for all patients
Slide 2: history + examination + investigations.

1.N3. a — Colonic diverticulitis
Diverticulitis is a LEFT lower abdominal cause (slides 4–5).

1.N4. a — True
Listed under upper abdominal and medical causes.

1.N5. a — Mesenteric adenitis
RLQ: appendicitis, mesenteric adenitis, right ureteric colic, right ovarian cyst rupture/torsion.

1.N6. c — Left lower abdominal pain

1.N7. d — Acute appendicitis
Medical causes: gastroenteritis, FMF, uremia, MI, DKA.

1.N8. a — True

1.N9. c — Acute appendicitis
Slide 8.

1.N10. a — The second and third decades

1.N11. b — Wide-mouthed and well drained
In the elderly it is rare because the lumen is almost obliterated.

1.N12. d — 2/3 of cases

1.N13. a — Faecolith
Slide 10.

1.N14. b — False
Non-obstructive (catarrhal) is LESS likely to perforate; obstructive tends to be suppurative/gangrenous.

1.N15. b — 3–5 days
Slide 11 (course).

1.N16. b — Thrombophlebitis of the portal venous system with hepatic abscess

1.N17. a — Malignant transformation

1.N18. b — Alvarado score (MANTRELS)

1.N19. d — Tenderness in the right lower quadrant and leucocytosis
Total score = 10.

1.N20. b — False
Pain ALWAYS occurs before vomiting.

1.N21. b — Starts around the umbilicus then shifts to the right lower abdomen
Migrating pain.

1.N22. b — False
Absence of leucocytosis does NOT exclude the diagnosis.

1.N23. a — Deep palpation of the left lower quadrant

1.N24. c — Extension of the right thigh with the patient lying on the left side

1.N25. b — Perforation

1.N26. c — Little tenderness, late diagnosis and low immunity
Slide 18.

1.N27. d — Pain displaced upwards
Pain displaced upwards is a feature in PREGNANCY.

1.N28. b — Pyelitis
High incidence of abortion or premature labour.

1.N29. b — Tenderness that shifts with change of position, usually in children

1.N30. c — After exploration

1.N31. d — Clinical

1.N32. a — Young healthy adults with appendicular mass
Appendicular mass → conservative (Ochsner–Sherren).

1.N33. a — Ochsner–Sherren regimen then interval appendectomy after 3 months

1.N34. c — Sonar-guided aspiration, surgical drainage if failed
Abscess: persistent pain, fever, tachycardia, increasing mass; US confirms.

1.N35. c — 98%

1.N36. b — Obstruction of the cystic duct or neck of the gallbladder by a stone

1.N37. b — More than 6 hours
Initially diffuse colicky upper abdominal, then right hypochondrium, may radiate to the right shoulder.

1.N38. a — Acute cholecystitis

1.N39. d — Normal

1.N40. a — Air under the diaphragm
Air under diaphragm = perforated viscus.

1.N41. a — 3 days
Or conservative then cholecystectomy after 6 weeks (to allow adhesions to subside).

1.N42. c — Surgery: cholecystectomy or cholecystostomy

1.N43. a — NSAIDs or corticosteroids

1.N44. b — 6:1

1.N45. c — Perforated peptic ulcer

1.N46. d — Erect chest X-ray showing air under the diaphragm
Gastrografin meal shows leakage of contrast.

1.N47. b — Graham's omental patch

1.N48. b — Lack of fibre leading to chronic constipation and increased intracolonic pressure

1.N49. c — Sigmoid colon

1.N50. b — False
CT with oral and IV contrast; barium enema is NOT done in the acute attack.

1.N51. a — Exploration with Hartmann's procedure
Pericolic abscess → U/S guided drainage.

1.N52. c — Strangulation intestinal obstruction

1.N53. b — Atrial fibrillation or mural thrombus after MI

1.N54. d — Maximum symptoms with minimal signs
Until peritonitis occurs.

1.N55. d — CT angiography

1.N56. b — 24 hours
Viable bowel: embolectomy (embolism), bypass graft (thrombosis), anticoagulation (venous).

1.N57. b — Anticoagulation therapy

1.N58. c — Stones of the urinary tract
Slide 33.

1.N59. d — Air under the diaphragm

2.1. c — Bile duct stone
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.2. a — Conservative treatment
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.3. d — Serum lipase level
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.4. a — Indicated in all cases
إجابة مقترحة (المصدر مفيهوش إجابة رسمية). معظم الـ pseudocysts بتختفي لوحدها، والتصريف بيكون للكيس اللي فضل أكتر من 6 أسابيع أو كبير أو حصله infection.

2.N1. b — Upper abdominal retroperitoneal organ
Slide 2.

2.N2. b — 2nd part of duodenum

2.N3. b — 1–2 litres
Alkaline enzyme-rich juice (amylase, lipase).

2.N4. d — Bile duct stones
Bile duct stones 50%, alcohol 30%, idiopathic 20%.

2.N5. b — 30%

2.N6. c — Hypothyroidism

2.N7. a — True
Trauma: accidental, operative or during ERCP.

2.N8. b — Premature activation of pancreatic enzymes inside the gland leading to autodigestion

2.N9. d — SIRS

2.N10. c — Fatty acids binding calcium forming calcium soaps (fat necrosis)

2.N11. b — Epigastric pain radiating to the back, acute onset and gradually increasing

2.N12. d — Leaning forward (prayer position)

2.N13. a — Retroperitoneal

2.N14. a — Bruising around the umbilicus

2.N15. d — Bruising in the loin (flank)

2.N16. a — Pancreatic pseudocyst

2.N17. d — Pancreatic abscess
Pancreatic abscess and pseudocyst are LOCAL complications.

2.N18. a — Hypovolemia

2.N19. c — Hypocalcemia

2.N20. a — Acute appendicitis
DD: perforated PU, acute cholecystitis, MVO, leaking aortic aneurysm, MI.

2.N21. c — 1000 IU
Remains elevated for 2–3 days.

2.N22. b — False
Not specific — may rise in perforated peptic ulcer (usually < 500 IU).

2.N23. b — Lipase

2.N24. b — Detect patients who will need mechanical ventilation

2.N25. c — Hypoglycemia

2.N26. b — Gallbladder stones and dilated biliary tree

2.N27. b — CT scan with IV contrast

2.N28. a — CBD stones are suspected

2.N29. c — Conservative treatment (R regimen)

2.N30. b — Counteract spasm of the sphincter of Oddi

2.N31. a — Keeping the patient NPO
New concept: IV fluids until nausea and vomiting stop.